INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Sacituzumab govitecan (Trodelvy)

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Regimen source
NCCP 00794
Version 3

21-day cycle

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

To control the cancer, relieve or delay symptoms, and help people live longer. This advanced-disease regimen does not promise a cure or an individual duration of benefit.

TROP-2-DIRECTED ANTIBODY–DRUG CONJUGATE

Sacituzumab govitecan (Trodelvy)

Carries the topoisomerase-I inhibitor SN-38 using an antibody that recognises Trop-2

Antibody recognises Trop-2 → SN-38 payload is released → deoxyribonucleic acid (the cell's genetic material) (DNA) copying is disrupted

The antibody component recognises Trop-2 on cancer cells. Its linked SN-38 payload interferes with topoisomerase I, an enzyme involved in managing DNA during copying, which can damage dividing cells. Healthy tissues can also be affected, particularly blood-forming cells and the gut; being antibody-directed does not mean it is free of chemotherapy-type effects.

The most important early problems to act on are fever or infection with low neutrophils, diarrhoea and dehydration, persistent nausea/vomiting, and infusion reactions. Report symptoms early rather than waiting for a planned scan or clinic.

The exact treatment being discussed

Who this covers

NCCP00794a covers adults with unresectable or metastatic triple-negative breast cancer after at least two prior systemic therapies, including at least one for advanced disease. The NCCP entry specifies ECOG 0–1. Broader European licensed indications must be checked separately against the applicable regimen and reimbursement pathway.

NCCP 00794 v3 · official regimen · Trodelvy EU Summary of Product Characteristics, sections 4.2–4.6 and 5.1; checked 20 October 2026 · retrieved 25 October 2026

Dose and administration

10 mg/kg intravenously on days 1 and 8 of a 21-day cycle; prescribed reductions may apply. The first infusion takes about three hours; later infusions usually take one to two hours if the earlier infusion was tolerated. Premedicines, checks and at least 30 minutes of observation after every infusion add to the visit. No take-home infusion pump is part of this regimen.

Duration and decisions

Treatment continues while it is helping and side effects remain acceptable. There is no fixed number of cycles; the oncology team reviews benefit, scans and tolerability.

Individual safety and source differences

Blood-count rules differ between Day 1 and Day 8. Known reduced UGT1A1 activity may increase side effects, but routine genotype testing is not required by the EU product information. Important differences between the NCCP and current EU dose-modification wording are flagged for Consultant and oncology-pharmacy review; patients should never change the dose themselves.

ASCENT evidence in previously treated triple-negative breast cancer

This explains the study setting and its meaning. Ask the team how the evidence applies to you.

The primary analysis excluded baseline brain metastases; the overall-population estimates differ. Open-label design, selected trial fitness and previous-treatment requirements limit extrapolation. Group medians do not predict an individual patient's benefit. This trial does not establish eligibility for every broader current EU indication.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact the oncology service immediately for fever, shivering or feeling acutely unwell; significant diarrhoea, inability to keep fluids down, dizziness, very little urine, or abdominal pain. Do not wait for the next treatment visit. During an infusion, tell staff immediately about breathing difficulty, swelling, rash, chest discomfort or faintness. Use the emergency service for severe breathing difficulty, collapse or other life-threatening symptoms.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Reference timing, not booked appointments. Ask who requests each test, when it is due, who reviews the result and whom to contact if it is missing. Delays can change cycle numbers; time-based monitoring remains time-based.

Check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Full blood count / neutrophils	Baseline blood count	Check before Day 1 and assess ANC before Day 8; the thresholds differ	Repeat the Day 1 / Day 8 assessment each cycle; extra counts if low or unwell	Continue pre-cycle count and Day 8 ANC assessment	Repeat until recovery if treatment is held; agree restart with the team
Kidney and liver tests	Baseline renal and liver profile	Before the cycle; repeat sooner for dehydration or symptoms	Before every cycle	Before every cycle; clinician-directed additional tests	Reassess any abnormality before changing or restarting treatment
Glucose / electrolytes	Glucose, electrolytes, magnesium, calcium, phosphate	Baseline results reviewed; repeat if diarrhoea or dehydration	Every second cycle under NCCP; sooner if clinically needed	Every second cycle, with symptom-driven additions	Recheck and replace losses after significant diarrhoea/ vomiting
Infusion observation	Review allergy / reaction history and premedicine plan	First infusion about 3 hours, then ≥30-minute observation	Later infusions 1–2 hours if tolerated; ≥30-minute observation each time	Continue observation at every infusion	Any significant reaction changes the future infusion plan
electrocardiogram (ECG) / echo	ECG if clinically indicated; no routine drug-specific echo	Repeat only if clinically indicated	Add testing for symptoms, risk factors or other medicines	No fixed echo schedule for sacituzumab alone	Investigate new cardiac symptoms appropriately
Pregnancy / fertility	Pregnancy test where relevant; fertility discussion and contraception	Repeat pregnancy test before the cycle where relevant	Pregnancy test before each cycle where relevant	Continue contraception and pre-cycle testing where relevant	Contraception continues for the specified period after the final dose

Check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Response imaging / markers	Record measurable sites and baseline imaging	Agree first response review	Usually every 2–4 months; individualise	Adapt to disease pace; earlier for symptoms	Markers supplement, never replace, assessment

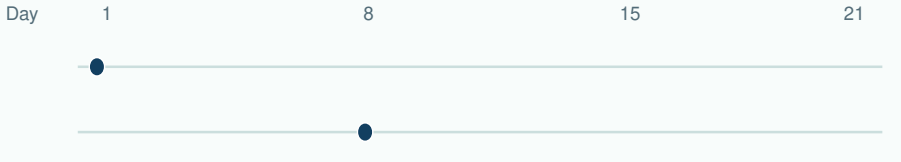
The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00794 v3 · official regimen · Trodelvy EU Summary of Product Characteristics, sections 4.2–4.6 and 5.1; checked 20 October 2026 · retrieved 25 October 2026 · Pan-Asian adapted ESMO early breast cancer guidelines · Pan-Asian adapted ESMO metastatic breast cancer guidelines · ESO-ESMO ABC 5 consensus: tumour-marker interpretation

THE WHOLE TREATMENT JOURNEY

Sacituzumab govitecan (Trodelvy): treatment and checks

Sacituzumab govitecan (Trodelvy)

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Previously treated unresectable or metastatic triple-negative breast cancer

Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Treatment NCCP 00794	Confirm exact indication and biomarkers	10 mg/kg intravenously on days 1 and 8 of a 21-day cycle; prescribed reductions may apply	Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.	Continue with safety / benefit review	Treatment continues while it is helping and side effects remain acceptable. There is no fixed number of cycles; the oncology team reviews benefit, scans and tolerability.

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Surgery / radiotherapy ESMO_MBC2023	Only for an agreed local-control or symptom need	Teams decide sequence / any treatment interruption	No routine surgery or radiotherapy (RT) implied	Review any local-treatment effects	Reassess if goals or symptoms change
Safety checks NCCP 00794 · SMPC	Baseline bloods and individual risks	Early checks from the monitoring table	Regimen-specific checks continue	Review results before continuation	Resolve abnormalities / agree next plan
Scans / follow-up ESMO_MBC2023	Confirm baseline and treatment setting	Agree the next imaging milestone	Use the matching surveillance / response plan	Review benefit and symptoms	Continue setting-specific follow-up

What each check is for

1



Blood tests

Before the first treatment: full blood count, kidney and liver blood tests, glucose and electrolytes including magnesium, calcium and phosphate. Pregnancy testing is needed where relevant. ECG is added if clinically indicated. The team checks infection, medicines, previous low blood counts and whether preventive granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) injections are appropriate. Blood count, kidney and liver tests are checked before each cycle; the neutrophil count must also meet the separate Day 8 threshold before that infusion. Glucose and the listed electrolytes are checked every second cycle. Pregnancy testing is repeated before each cycle where relevant. Review diarrhoea, hydration, nausea, oral intake, fever and infection before each infusion. Scans follow the individual response-assessment plan.

Checks safety before continuation.

Ask / act: Agree the due window, booking team and result reviewer. Contact the team if a test is overdue.

2

Heart checks

An ECG is used when clinically indicated. Sacituzumab alone does not require a routine echocardiogram schedule. Cardiac symptoms or other medicines may change the plan.

Checks cardiac safety when the agent or clinical risk requires it.

Ask / act: Agree the due window, booking team and result reviewer. Contact the team if a test is overdue.

3

Imaging

Response assessment in advanced breast cancer is generally every two to four months, adapted to disease dynamics, sites, symptoms and treatment. Less frequent monitoring may suit indolent disease. New symptoms or suspected progression need earlier assessment. The team chooses the imaging method; this is not an automatic three-month computed tomography (CT) chest/abdomen/pelvis order.

Uses the correct surveillance or response-assessment pathway.

Ask / **act:** Agree the due window, booking team and result reviewer. Contact the team if a test is overdue.

4

Bone health

A routine dual-energy X-ray absorptiometry (bone density) (DXA) scan is not a sacituzumab-specific requirement. Bone health, bone metastases and any bone-strengthening treatment need their own individual plan.

Companion endocrine therapy and individual fracture risk may change the plan.

Ask / **act:** Agree the due window, booking team and result reviewer. Contact the team if a test is overdue.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks throughout treatment

Blood tests assess treatment safety. Scans assess disease or investigate symptoms. Echo and ECG answer different questions.

Before treatment

Before the first treatment: full blood count, kidney and liver blood tests, glucose and electrolytes including magnesium, calcium and phosphate. Pregnancy testing is needed where relevant. ECG is added if clinically indicated. The team checks infection, medicines, previous low blood counts and whether preventive G-CSF injections are appropriate.

During treatment

Blood count, kidney and liver tests are checked before each cycle; the neutrophil count must also meet the separate Day 8 threshold before that infusion. Glucose and the listed electrolytes are checked every second cycle. Pregnancy testing is repeated before each cycle where relevant. Review diarrhoea, hydration, nausea, oral intake, fever and infection before each infusion. Scans follow the individual response-assessment plan.

Heart checks

An ECG is used when clinically indicated. Sacituzumab alone does not require a routine echocardiogram schedule. Cardiac symptoms or other medicines may change the plan.

Bone health

A routine DXA scan is not a sacituzumab-specific requirement. Bone health, bone metastases and any bone-strengthening treatment need their own individual plan.

Your wider treatment plan

Additional treatments depend on the confirmed setting and indication.

Advanced-disease scans

Response assessment in advanced breast cancer is generally every two to four months, adapted to disease dynamics, sites, symptoms and treatment. Less frequent monitoring may suit indolent disease. New symptoms or suspected progression need earlier assessment. The team chooses the imaging method; this is not an automatic three-month CT chest/abdomen/pelvis order.

Tumour markers

If informative, particularly when elevated at baseline or disease is not readily measurable, tumour markers may supplement clinical assessment and imaging. A marker rise alone should not trigger a treatment change. There is no universal fixed marker timetable.

Pregnancy, fertility and contraception

Discuss fertility before treatment. Effective contraception is needed during treatment and for six months after the final dose for women who could become pregnant, and for three months for male patients with female partners who could become pregnant. Do not breastfeed during treatment; the EU product information also advises against breastfeeding for one month after the last dose.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day 1 infusion	 **Day 1** Blood review, premedicines, infusion and observation
Day 8 infusion	 **Day 8** Reassess symptoms and ANC, then premedicines, infusion and observation
Nausea / bowel symptoms	 **Days 1–21** May occur after either dose or between visits. This is a watch window, not a predicted symptom period.
Blood-count / infection watch	 **Days 1–21** Risk varies across the cycle; fever or feeling unwell needs urgent advice on any day.
Energy / recovery	 **Days 1–21** Fatigue may fluctuate or build over repeated cycles. No planned infusion on Day 15 does not guarantee absence of side effects.

Treatment and rest: Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.

Tests and appointments: Use the monitoring table for baseline and later checks. A rest week does not remove infection or other risks.

> **Why these bands stay broad:** Treatment days are specified by the regimen. Symptom timing varies; the full-cycle symptom bands show that an effect can occur at any point, not a measured probability or severity. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00794 v3 · official regimen ↗ · Trodelvy EU Summary of Product Characteristics, sections 4.2–4.6 and 5.1; checked 20 October 2026 · retrieved 25 October 2026 ↗

There are no guaranteed "safe days". Contact the oncology service immediately for fever, shivering or feeling acutely unwell; significant diarrhoea, inability to keep fluids down, dizziness, very little urine, or abdominal pain. Do not wait for the next treatment visit. During an infusion, tell staff immediately about breathing difficulty, swelling, rash, chest discomfort or faintness. Use the emergency service for severe breathing difficulty, collapse or other life-threatening symptoms.

Practical support

Prevent nausea before both doses

This is a high-emetic-risk regimen. Preventive antiemetics are needed for both Day 1 and Day 8, with take-home medicines and clear instructions. The steroid used as an infusion premedicine also provides antiemetic cover, so the team must reconcile the total dose.

A written diarrhoea plan

Report diarrhoea early. The team assesses infection, dehydration and electrolyte loss, and supplies an appropriate antidiarrhoeal prescription. Follow the written oncology instructions; do not improvise repeated high doses of loperamide.

Blood-count support

G-CSF injections may be considered from cycle 1 if the risk of febrile neutropenia is increased, or after significant neutropenia. Product and timing depend on the two-infusion schedule and the prescribing plan.

Infusion-reaction prevention

The unit gives premedicines to reduce reaction risk and observes you during and after each infusion. Tell staff immediately if you feel different or unwell.

Early cramping or diarrhoea during an infusion

This can be an acute cholinergic effect and needs staff assessment. Atropine may be used for later infusions when appropriate; tell the team if you have glaucoma.

Taking tablets / attending treatment

Practical treatment details

Cannula or existing venous access

The team chooses suitable intravenous access.

A port is not automatically required.

If you already have a port, staff access and flush it according to local policy; report pain, redness or swelling.

Before leaving the unit

If granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is prescribed, ask exactly which days to take it around both treatment visits.

Questions to ask

When do I take or receive treatment?

Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate. The first infusion takes about three hours; later infusions usually take one to two hours if the earlier infusion was tolerated. Premedicines, checks and at least 30 minutes of observation after every infusion add to the visit. No take-home infusion pump is part of this regimen.

When are bloods, ECGs or echoes needed?

Before the first treatment: full blood count, kidney and liver blood tests, glucose and electrolytes including magnesium, calcium and phosphate. Pregnancy testing is needed where relevant. ECG is added if clinically indicated. The team checks infection, medicines, previous low blood counts and whether preventive G-CSF injections are appropriate. Blood count, kidney and liver tests are checked before each cycle; the neutrophil count must also meet the separate Day 8 threshold before that infusion. Glucose and the listed electrolytes are checked every second cycle. Pregnancy testing is repeated before each cycle where relevant. Review diarrhoea, hydration, nausea, oral intake, fever and infection before each infusion. Scans follow the individual response-assessment plan. An ECG is used when clinically indicated. Sacituzumab alone does not require a routine echocardiogram schedule. Cardiac symptoms or other medicines may change the plan.

Explain the plan back: Which medicine is taken or given when, what checks are expected next, and which symptoms mean you should call today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Sacituzumab govitecan (Trodelvy)

Active medicines: Sacituzumab govitecan (Trodelvy)

Emetogenic risk: High · sacituzumab govitecan

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 before treatment
Ondansetron	16 mg orally	Day 1 before treatment Review QT risk, hepatic impairment, constipation and interacting medicines.
Dexamethasone	12 mg orally	Day 1 before treatment Reconcile all steroid premedication; do not duplicate.

Medicine	Dose / route	When
Olanzapine	5 mg orally once daily	Days 1–4 Review sedation, falls, glucose and QT risk. Avoid routine combination with metoclopramide because of additive extrapyramidal effects; select rescue individually.
Aprepitant	80 mg orally once daily	Days 2–3
Dexamethasone	8 mg orally once daily	Days 2–4 Confirm suitability and total steroid exposure.
Metoclopramide	10 mg orally up to three times daily	Rescue use / duration to be completed by prescriber Review renal function and neurological contraindications; avoid automatic prolonged use.

Additional phase-specific support

- This is a high-emetic-risk regimen. Preventive antiemetics are needed for both Day 1 and Day 8, with take-home medicines and clear instructions. The steroid used as an infusion premedicine also provides antiemetic cover, so the team must reconcile the total dose.
- Report diarrhoea early. The team assesses infection, dehydration and electrolyte loss, and supplies an appropriate antidiarrhoeal prescription. Follow the written oncology instructions; do not improvise repeated high doses of loperamide.
- granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) injections may be considered from cycle 1 if the risk of febrile neutropenia is increased, or after significant neutropenia. Product and timing depend on the two-infusion schedule and the prescribing plan.
- The unit gives premedicines to reduce reaction risk and observes you during and after each infusion. Tell staff immediately if you feel different or unwell.
- This can be an acute cholinergic effect and needs staff assessment. Atropine may be used for later infusions when appropriate; tell the team if you have glaucoma.
- This is the high-risk national antiemetic template anchored to this infusion day. Day8 repeats prophylaxis only if treatment proceeds; do not print both scripts as simultaneous orders. Antihistamine/antipyretic infusion premedication and G-CSF are separate regimen-specific decisions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · Sacituzumab govitecan (Trodelvy)

Active medicines: Sacituzumab govitecan (Trodelvy)

Emetogenic risk: High · sacituzumab govitecan

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 8 before treatment

Medicine	Dose / route	When
Ondansetron	16 mg orally	Day 8 before treatment Review QT risk, hepatic impairment, constipation and interacting medicines.
Dexamethasone	12 mg orally	Day 8 before treatment Reconcile all steroid premedication; do not duplicate.
Olanzapine	5 mg orally once daily	Days 8–11 Review sedation, falls, glucose and QT risk. Avoid routine combination with metoclopramide because of additive extrapyramidal effects; select rescue individually.
Aprepitant	80 mg orally once daily	Days 9–10
Dexamethasone	8 mg orally once daily	Days 9–11 Confirm suitability and total steroid exposure.
Metoclopramide	10 mg orally up to three times daily	Rescue use / duration to be completed by prescriber Review renal function and neurological contraindications; avoid automatic prolonged use.

Additional phase-specific support

- This is a high-emetic-risk regimen. Preventive antiemetics are needed for both Day 1 and Day 8, with take-home medicines and clear instructions. The steroid used as an infusion premedicine also provides antiemetic cover, so the team must reconcile the total dose.
- Report diarrhoea early. The team assesses infection, dehydration and electrolyte loss, and supplies an appropriate antidiarrhoeal prescription. Follow the written oncology instructions; do not improvise repeated high doses of loperamide.
- granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) injections may be considered from cycle 1 if the risk of febrile neutropenia is increased, or after significant neutropenia. Product and timing depend on the two-infusion schedule and the prescribing plan.
- The unit gives premedicines to reduce reaction risk and observes you during and after each infusion. Tell staff immediately if you feel different or unwell.
- This can be an acute cholinergic effect and needs staff assessment. Atropine may be used for later infusions when appropriate; tell the team if you have glaucoma.
- This is the high-risk national antiemetic template anchored to this infusion day. Day8 repeats prophylaxis only if treatment proceeds; do not print both scripts as simultaneous orders. Antihistamine/antipyretic infusion premedication and G-CSF are separate regimen-specific decisions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sacituzumab govitecan (Trodelvy)

Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.

Antibody–drug conjugate · days 1 and 8 every 3 weeks · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Low blood counts / infection

Sacituzumab govitecan

Fever, shivering or suddenly feeling unwell needs immediate oncology advice; a low neutrophil count itself may cause no symptoms.

2 Diarrhoea / dehydration

Sacituzumab govitecan

Report loose stools early, especially with cramps, fever, dizziness or reduced urine. Follow the prescribed diarrhoea plan.

3 Nausea / vomiting

Sacituzumab govitecan

Use preventive medicines for both infusion days. Contact the team if you cannot keep fluids down.

4 Fatigue / anaemia

Sacituzumab govitecan

Tell the team about limiting tiredness, breathlessness or dizziness; blood tests help identify anaemia and other causes.

5 Hair loss

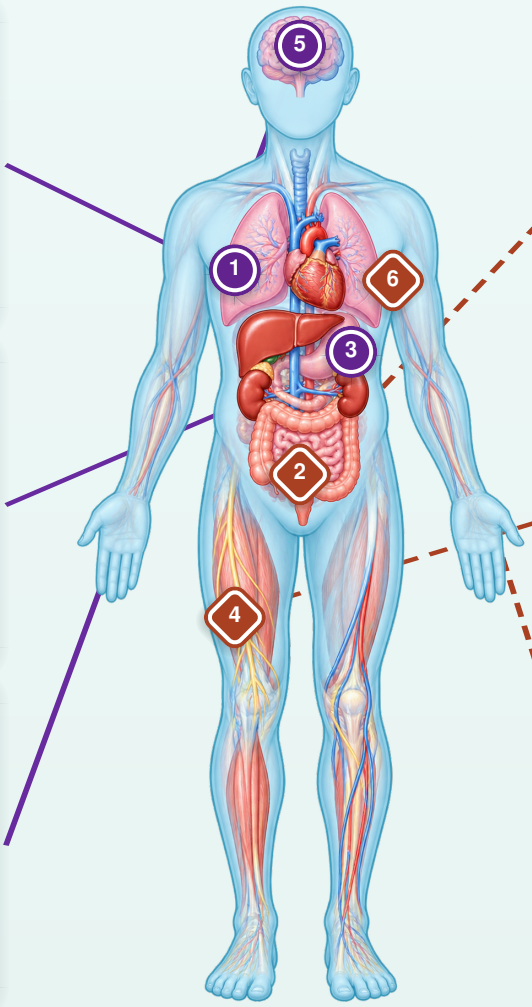
Sacituzumab govitecan

Hair thinning or loss can occur; ask about practical scalp and appearance support.

6 Infusion reaction

Sacituzumab govitecan

Tell staff immediately about rash, swelling, breathing difficulty or faintness during treatment.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology service immediately for fever, shivering or feeling acutely unwell; significant diarrhoea, inability to keep fluids down, dizziness, very little urine, or abdominal pain. Do not wait for the next treatment visit. During an infusion, tell staff immediately about breathing difficulty, swelling, rash, chest discomfort or faintness. Use the emergency service for severe breathing difficulty, collapse or other life-threatening symptoms. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00794 v3 · official regimen ↗**
   https://healthservice.hse.ie/documents/6417/794_v3_Sacituzumab_Govitecan_Therapy.pdf
2. **Trodelvy EU Summary of Product Characteristics, sections 4.2–4.6 and 5.1; checked 20 October 2026 · retrieved 25 October 2026 ↗**
   https://www.ema.europa.eu/en/documents/product-information/trodelvy-epar-product-information_en.pdf
3. **Pan-Asian adapted ESMO early breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11145753/
4. **Pan-Asian adapted ESMO metastatic breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10186487/
5. **ESO-ESMO ABC 5 consensus: tumour-marker interpretation**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
6. **Bardia et al. Sacituzumab Govitecan in Metastatic Triple-Negative Breast Cancer. NEJM 2021;384:1529–1541**
   https://pubmed.ncbi.nlm.nih.gov/33882206/
7. **ASCENT final full-population report (2024)**
   https://pubmed.ncbi.nlm.nih.gov/38422473/
8. **NCCP antiemetics v8, Table 1 pp4–5**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf